INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin AUC 1.5 with radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00332
Version 4

Once every 7 days for 7 planned weekly doses, during radiotherapy.

Treatment stages

01 **Preparation and transition**
Team confirms readiness

02 **Carboplatin with radiotherapy**
7 weekly doses

03 **Recovery and further review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	16
04	Sources and further information	25

My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Carboplatin AUC 1.5 with radiotherapy
National Cancer Control Programme (NCCP) 00332

Head and neck cancer: weekly chemoradiation

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shivering, sudden illness, unusual bruising or bleeding, persistent vomiting, severe mouth or throat pain, inability to drink or much less urine. Report new hearing changes, numbness, worsening swallowing, choking or skin breakdown promptly. During an infusion, tell staff immediately about rash, flushing, chest tightness, breathing difficulty or faintness. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112 or 999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each part of treatment does

Carboplatin and radiotherapy are used together as part of the treatment plan for head and neck cancer. The team explains the treatment aim, expected benefit and why this medicine and schedule suit your situation.

PLATINUM CHEMOTHERAPY

Carboplatin

Carboplatin



Damages cancer-cell deoxyribonucleic acid (the cell’s genetic material) (DNA)

Carboplatin interferes with DNA repair and cell division. Its dose is calculated from kidney function. Blood counts can fall, including platelets that help stop bleeding. It is a different prescription from cisplatin. AUC is a prescribing target for drug exposure, not the number of milligrams in your bag; your clinical team calculates the dose using kidney function.

DNA damage → Cell division disrupted → Cancer growth reduced

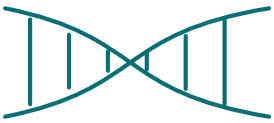
What to watch for

Report fever, unusual bruising or bleeding, marked tiredness or an infusion reaction.

LOCAL TREATMENT

Radiotherapy

Radiotherapy



Damages cancer cells in the planned treatment area

Radiotherapy directs radiation at a carefully planned area. It is delivered on its own appointment calendar. Effects depend on which tissues receive radiation and can build up during the course or continue after it finishes.

Treatment area planned → Repeated local treatment → Recovery and late-effect review

What to watch for

Report difficulty swallowing, weight loss, mouth pain, skin breakdown or problems managing saliva.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis

Keep your diagnosis, treatment name and agreed goal with your records.

Site, type and stage

Ask where the cancer started, what the biopsy shows and how far it has spread. Site, stage, previous treatment and general health affect the available options.

Which pathway applies?

Chemoradiation after TPF induction for locally advanced stage III or intravenous (IV) squamous cell cancer of the head and neck.

Tumour tests and medicine safety tests

Your team explains any HPV/p16, Epstein–Barr virus or other tumour tests appropriate to your cancer site. Not every site needs the same tests. These are different from blood, kidney and other medicine safety checks.

Understanding the evidence

The NCCP regimen combines evidence and specialist guidance for a defined treatment setting.

A treatment name such as TPF or chemoradiation does not make different dose schedules, patient groups or sequences interchangeable. Ask which research and clinical factors support your exact plan. No single study percentage is presented as your personal chance of benefit.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The stages of your treatment

Treatment stages

01 **Preparation and transition**
Team confirms readiness

02 **Carboplatin with radiotherapy**
7 weekly doses

03 **Recovery and further review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

TEAM CONFIRMS READINESS

Preparation and transition

Before this weekly course

This phase follows induction chemotherapy. The source uses different reference points for the 3–8 week transition interval, so the oncology and radiation teams must confirm the actual start date after reviewing response and recovery.

7 WEEKLY DOSES

Carboplatin with radiotherapy

Weekly doses 1–7

Carboplatin AUC 1.5 is given by intravenous (IV) infusion over 30 minutes once every 7 days for 7 planned weekly doses, only while radiotherapy is being given. It must be stopped if the radiation course is cut short. Radiation sessions follow their own calendar.

INDIVIDUAL PLAN



Recovery and further review

After chemoradiation

After the planned chemoradiation course, the team reviews recovery and response and decides whether surgery or other treatment is appropriate. Induction and this weekly phase have separate prescriptions; later treatment is not automatic.

This phase follows induction chemotherapy. The source uses different reference points for the 3–8 week transition interval, so the oncology and radiation teams must confirm the actual start date after reviewing response and recovery. After the planned chemoradiation course, the team reviews recovery and response and decides whether surgery or other treatment is appropriate. Induction and this weekly phase have separate prescriptions; later treatment is not automatic.

BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Routine blood safety checks are different from tests ordered for a symptom and from scans used to assess the cancer. Your team supplies exact dates and explains results.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Blood and organ tests	Before starting: full blood count, kidney and liver tests, plus measured kidney filtration (preferred) or an appropriate estimate for carboplatin dosing. Review prior treatment, medicines, hearing and nerve symptoms. Arrange dental, nutrition and swallowing preparation for head and neck radiation.	Full blood count and kidney and liver tests are required before every weekly cycle. Kidney results guide the carboplatin dose. Review infection or bleeding symptoms, mouth and skin pain, weight, intake and swallowing throughout radiation.	Report symptoms promptly; extra blood or organ tests may be needed.	The team reviews recovery, results and earlier side effects before another dose or phase.	Mouth, throat and skin symptoms can continue after the last session. The team follows blood-count recovery, nutrition, swallowing and any kidney, hearing or nerve problems. Keep dental care and the agreed radiation late-effect follow-up. Scans, examinations and any thyroid or other tests follow the individual treatment plan; there is no universal scan or tumour-marker timetable here.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment and appointments	Confirm the exact regimen, treatment aim, prescription and appointment calendar.	Carboplatin AUC 1.5 is infused through a vein over 30 minutes. The dose in milligrams is calculated by the treating team using kidney function. Allow extra time for assessment, preparation and prescribed anti-sickness treatment, as well as separate radiation appointments. This weekly carboplatin regimen does not automatically require the cisplatin hydration or docetaxel steroid schedule from an earlier phase.	Follow the written support plan and record symptoms and medicines taken.	After the planned chemoradiation course, the team reviews recovery and response and decides whether surgery or other treatment is appropriate. Induction and this weekly phase have separate prescriptions; later treatment is not automatic.	Record the final treatment dates and the named follow-up team.
Mouth, nutrition and swallowing	Agree oral care and assess weight, intake and swallowing. Arrange dental review before planned head and neck radiation.	Review mouth pain, weight, drinking, swallowing and, during radiation, the treated skin.	Report poor intake, choking, weight loss or inability to drink.	Dietitian and speech and language plans adapt to symptoms and recovery.	Continue dental, nutrition and swallowing care as needed.

Check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Tumour assessment and imaging	Confirm the cancer site, stage and tests relevant to that site.	Discuss symptoms and the agreed treatment aim.	Examinations or scans may be arranged for a particular concern.	Response, recovery and your preferences inform the next treatment decision.	Use your individual scan and clinic plan; this regimen does not set one universal scan or blood tumour-marker schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00332 v4: official regimen · NCI: head and neck cancers · NCI: oral complications of cancer therapies, patient version · NCI: nutrition during cancer treatment

Tests in your plan

Full blood count

WHEN

At baseline and before every weekly carboplatin cycle.

Checks infection-fighting white cells, red cells and platelets; low counts may require a delay or another change.

Kidney function and filtration assessment

WHEN

Before starting and before each weekly cycle. The team determines the required recency of creatinine results for the calculation method.

Carboplatin dosing depends on kidney function. Measured GFR is preferred where feasible, with an appropriate estimate used when needed.

Liver function

WHEN

At baseline and before every weekly cycle.

Supports safe treatment review and investigation of changes.

Hearing and nerve review

If part of your plan

WHEN

Review symptoms before dosing; hearing tests or further examination when indicated.

Identifies new problems and effects remaining from earlier platinum chemotherapy.

Dental, nutrition and swallowing assessment

WHEN

Prepare before head and neck radiation; reassess as symptoms or intake change.

Helps plan safe intake, mouth care and prevention of avoidable radiation-related complications.

Imaging and cancer assessment

If part of your plan

WHEN

Examinations and scans on the individual oncology and radiation plan.

Assesses the cancer and informs further treatment.

THE WHOLE TREATMENT JOURNEY

Carboplatin AUC 1.5 · weekly chemoradiation after induction: treatment and checks

Concurrent carboplatin and radiotherapy after induction for stage III/IV non-metastatic head and neck squamous cell carcinoma, subject to response and recovery.

Carboplatin AUC 1.5 IV every 7 days for 7 cycles, only while radiotherapy is being delivered. Discontinue carboplatin if radiotherapy is truncated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Treatment phase NCCP	Confirm the full multidisciplinary team (MDT) plan and individual treatment calendar	Carboplatin AUC 1.5 IV every 7 days for 7 cycles, only while radiotherapy is being delivered. Discontinue carboplatin if radiotherapy is truncated.	Attend planned radiotherapy and report problems early	The treatment paragraph describes a start 3–8 weeks (Days 22–56) after the START of cycle 3 of induction; the indication paragraph says 3–8 weeks after COMPLETION of induction. The MDT must resolve the reference point and set the actual radiotherapy and chemotherapy dates. Confirm the induction route and adequate recovery.	Record the completed medicines and next follow-up plan
Blood and organ checks NCCP	Full blood count, renal and liver profile; clinical status, medicines, allergy and previous treatment review.	Full blood count, renal and liver profile before each cycle; use the current regimen's exact count and organ-function rules.	Report infection, bleeding, reduced urine or dehydration	Repeat or reassess tests if toxicity or recovery changes the plan	Further blood tests and imaging are arranged by the treating team

Care / check	Before starting	Treatment visit	Between visits	Dose / phase review	After treatment
Support medicines NCCP	Confirm the moderate-risk antiemetic prescription and rescue plan	No routine docetaxel premedication or cisplatin hydration belongs to this phase	NCCP gives no routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or additional supportive-care prescription for this weekly phase. Do not carry over induction G-CSF, docetaxel premedication or cisplatin hydration automatically.	Reconcile all doses, steroid totals and start/stop dates when the phase changes	Review ongoing medicine need with the team
Mouth, nutrition and swallowing NCCP	Assess mouth, teeth, weight and swallowing needs	Review pain, mouth symptoms, oral intake and hydration	Report inability to drink, severe soreness or weight loss	Dietetic, swallowing and dental support according to need	Continue rehabilitation and late-effect care where needed
Urgent contact NCCP	Save the unit's 24-hour contact and fever instructions	Tell staff about new symptoms or reactions	Seek urgent advice for infection, bleeding, chest pain, severe diarrhoea/ vomiting or inability to drink	Do not wait for the next routine appointment if acutely unwell	Keep follow-up and urgent-contact instructions available

What each check is for

1 Concurrent chemoradiation

Follow your agreed treatment calendar

Give carboplatin during radiotherapy as part of the planned combined treatment after induction. Carboplatin is a radiation partner in this phase; it is not a continuation of the TPF medicines.

Ask / **act:** Confirm active medicines, written supportive-care plan and review criteria.

2

Next-phase / completion review

After the planned course and recovery review

The team checks response and fitness before the next treatment or follow-up stage.

Ask / act: Ask the team to confirm the next phase and actual dates.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first visit

Confirm the exact regimen, treatment aim, baseline tests, transport, support medicines and verified urgent contact numbers. Bring a full medicine list and any hearing, nerve or swallowing concerns.

STEP 2

Time in the unit

Carboplatin AUC 1.5 is infused through a vein over 30 minutes. The dose in milligrams is calculated by the treating team using kidney function. Allow extra time for assessment, preparation and prescribed anti-sickness treatment, as well as separate radiation appointments. This weekly carboplatin regimen does not automatically require the cisplatin hydration or docetaxel steroid schedule from an earlier phase.

STEP 3

Keeping appointments together

Keep one calendar for blood tests, treatment and any radiation, dental, nutrition or swallowing appointments. If an appointment is cancelled, ask the relevant teams to agree the revised plan.

STEP 4

Practical help

Tell the team about travel, cost, work, caring responsibilities, communication or reading needs. Ask whether a support person or interpreter can attend and how treatment changes will be explained.

STEP 5

Leaving the unit

Check the next blood-test and treatment dates, medicine instructions and whom to call if unwell. Have the service confirm the daytime, out-of-hours and backup contact routes.

Recovery, assessment and longer-term care

The last treatment and the end of recovery are different milestones.

Recovery and continuing effects

Mouth, throat and skin symptoms can continue after the last session. The team follows blood-count recovery, nutrition, swallowing and any kidney, hearing or nerve problems. Keep dental care and the agreed radiation late-effect follow-up. Scans, examinations and any thyroid or other tests follow the individual treatment plan; there is no universal scan or tumour-marker timetable here.

Cancer assessment

Your team sets the examination and imaging plan for your cancer and symptoms. Record who arranges each test and who will explain the result. New or worsening symptoms should be reported rather than saved for the next routine scan.

Information for other clinicians

Keep the regimen name, treatment dates, ongoing medicines and relevant alert information. Tell future clinicians and dentists about head and neck radiation if you receive it, and about any persistent kidney, hearing, nerve or swallowing problem.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Carboplatin AUC 1.5 is given by IV infusion over 30 minutes once every 7 days for 7 planned weekly doses, only while radiotherapy is being given. It must be stopped if the radiation course is cut short. Radiation sessions follow their own calendar.

Treatment or check	Timing in this view
Weekly carboplatin	 **Day 1** AUC 1.5 by IV infusion over 30 minutes, if the team confirms treatment can proceed.
Radiation calendar	 **Days 1–7** Attend the radiation sessions listed by your radiation team. This band does not mean radiation is delivered on every day of the week.
Symptoms and support	 **Days 1–7** Symptoms can build up during treatment and continue afterwards. Call promptly if unwell.

Treatment sequence: Carboplatin AUC 1.5 is given by IV infusion over 30 minutes once every 7 days for 7 planned weekly doses, only while radiotherapy is being given. It must be stopped if the radiation course is cut short. Radiation sessions follow their own calendar. After the planned chemoradiation course, the team reviews recovery and response and decides whether surgery or other treatment is appropriate. Induction and this weekly phase have separate prescriptions; later treatment is not automatic.

Baseline and ongoing checks: Before starting: full blood count, kidney and liver tests, plus measured kidney filtration (preferred) or an appropriate estimate for carboplatin dosing. Review prior treatment, medicines, hearing and nerve symptoms. Arrange dental, nutrition and swallowing preparation for head and neck radiation. Full blood count and kidney and liver tests are required before every weekly cycle. Kidney results guide the carboplatin dose. Review infection or bleeding symptoms, mouth and skin pain, weight, intake and swallowing throughout radiation.

Appointments and administration: Carboplatin AUC 1.5 is infused through a vein over 30 minutes. The dose in milligrams is calculated by the treating team using kidney function. Allow extra time for assessment, preparation and prescribed anti-sickness treatment, as well as separate radiation appointments. This weekly carboplatin regimen does not automatically require the cisplatin hydration or docetaxel steroid schedule from an earlier phase.

Recovery and follow-up: Mouth, throat and skin symptoms can continue after the last session. The team follows blood-count recovery, nutrition, swallowing and any kidney, hearing or nerve problems. Keep dental care and the agreed radiation late-effect follow-up. Scans, examinations and any thyroid or other tests follow the individual treatment plan; there is no universal scan or tumour-marker timetable here.

> **Why these bands stay broad:** This is one reference week of the concurrent phase, not a complete radiation calendar. If a radiation appointment changes, contact both teams before the next carboplatin dose; do not assume chemotherapy can

proceed without the radiation course. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00332 v4: official regimen ↗

There are no guaranteed "safe days". Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Weekly medicine visit

Blood-test results, symptoms and the radiation plan are reviewed before carboplatin. The infusion takes about 30 minutes, but the whole appointment also includes assessment, preparation and any prescribed support medicines. Tell staff about new symptoms or any radiation cancellation before the infusion.

Day 2–7: Radiation appointments and symptom care

Attend radiation on its own calendar, follow the mouth, skin, nutrition and swallowing plan, and record symptoms. The weekly chemotherapy cycle does not imply seven daily radiation appointments. Call if eating, drinking, swallowing or pain control becomes difficult, or if you are unwell. Keep the next blood-test appointment.

Support around the medicines

1 A new support plan for this phase

Have one written list of preventive, rescue and regular medicines. Do not automatically continue steroid premedication, growth factor, antibiotics or cisplatin fluids from an earlier regimen; each requires a current prescription.

2 Anti-sickness treatment

The team prescribes anti-sickness medicines according to this carboplatin schedule and local policy. Ask which medicines prevent symptoms, which are rescue doses and when persistent vomiting requires a call.

3 Mouth and dental care

Arrange dental preparation before head and neck radiation. Follow the prescribed brushing, fluoride, rinse and pain-relief plan. Report ulcers, infection, bleeding or difficulty opening the mouth. Tell future dentists about radiation before invasive treatment.

4 Nutrition and hydration

A dietitian can adapt food, supplements or feeding support to your needs. Ask for a practical fluid plan that accounts for swallowing and heart or kidney problems. Call early for weight loss, poor intake or inability to drink.

5 Swallowing and communication

Follow advice from your speech and language therapist on safe swallowing, exercises and textures. Tell the team about choking, a wet voice after swallowing, food sticking, hearing difficulty or communication needs.

6 Skin and symptom relief

Use the radiation team’s skin and pain plan. Symptoms may increase during the course or persist after it ends; ask for review when care is no longer controlling them.

7 Fertility, pregnancy and other medicines

Discuss fertility, pregnancy, breastfeeding and the medicine-specific contraception period. Bring medicines, supplements and allergies and check before starting new products.

Treatment choices and your priorities

Discuss the whole plan and each stage before consenting.

Benefit and alternatives

Carboplatin and radiotherapy are used together as part of the treatment plan for head and neck cancer. The team explains the treatment aim, expected benefit and why this medicine and schedule suit your situation. Ask how the expected benefit and burdens compare with suitable alternatives and with your priorities.

When treatment changes

The clinician may delay, change or stop treatment because of symptoms, test results, response or your preferences. Record which component is affected and what must happen before restarting. Do not restart from an old prescription.

Preparing for and receiving treatment

Plan the appointments and practical support around the exact prescription.

The treatment calendar

Carboplatin AUC 1.5 is given by IV infusion over 30 minutes once every 7 days for 7 planned weekly doses, only while radiotherapy is being given. It must be stopped if the radiation course is cut short. Radiation sessions follow their own calendar. This is one reference week of the concurrent phase, not a complete radiation calendar. If a radiation appointment changes, contact both teams before the next carboplatin dose; do not assume chemotherapy can proceed without the radiation course.

Recording what was given

Write the actual date, phase, medicines and any change. Record a delayed or omitted dose clearly; it is not automatically made up. The wallet card gives a brief record and the passport keeps the fuller history.

Between visits: symptoms and medicines

Use the written home plan and contact route.

Symptoms have no safe-day guarantee

Effects may begin during treatment or develop later, and some build up over repeated doses. Call for a new, severe or worsening symptom even when a calendar calls the day a recovery day.

One medicine list

Keep prevention medicines, rescue medicines and regular medicines together on one reconciled written list. Record what you take and whether it helps. Do not double a missed dose or reuse an old prescription unless your treating team instructs you.

Mouth, teeth and jaw care

Dental preparation and gentle daily care reduce avoidable problems.

Before treatment

Arrange dental assessment before head and neck radiation. The dental and oncology teams decide what treatment is needed and how much healing time is required; do not schedule extractions independently.

During treatment

Follow the prescribed brushing, fluoride, rinse and pain-relief advice. Report ulcers, infection, new bleeding, difficulty wearing dentures or worsening jaw opening. Avoid starting a new mouthwash without checking if the mouth is sore.

After head and neck radiation

Dry mouth can increase dental problems and some radiation effects last. Keep regular dental care and tell any future dentist that you have had head and neck radiation before invasive treatment. Your radiation/dental team plans any procedure.

Eating, drinking and maintaining strength

The aim is adequate intake and comfort during treatment.

Recognising an intake problem

Record weight and report food or drink becoming harder to manage. Pain, taste change, dry mouth or nausea can each need a different response; a dietitian can adapt the plan to you.

If ordinary food is not enough

The team may suggest food fortification, supplements or tube feeding when intake or swallowing safety requires it. A tube is a support option, not a judgement about how well you have tried to eat. Training and follow-up are needed.

Fluid needs are individual

Ask for a realistic drinking plan that accounts for kidney/heart problems and any feeding tube. Persistent vomiting, dizziness, markedly reduced urine or inability to drink needs urgent contact.

Swallowing, voice and communication

A speech and language therapist can help protect function.

Swallowing assessment

Report coughing or choking with food or drink, a wet-sounding voice after swallowing, food sticking or repeated chest infections. The team may need to assess aspiration risk and safe textures.

Exercises and rehabilitation

Do swallowing or jaw exercises only as taught for your situation. Treatment and recovery can change what is comfortable and safe. Ask for reassessment when symptoms change.

Communication needs

Tell the team about changes in speech, voice or hearing. Ask for written instructions, communication aids, an interpreter or help involving a support person.

Daily life and emotional support

Practical adjustments can make the plan easier to follow.

Energy and activity

Balance rest with manageable activity according to your ability and team advice. Ask for help if weakness, falls, sleep difficulty or pain interferes with daily life.

Smoking and alcohol

Ask for support to stop smoking and discuss alcohol with the team. Mouth irritation, medicine interactions and head and neck recovery can change what is safe for you.

Emotional and practical concerns

Tell the team about anxiety, appearance or intimacy concerns, finances, travel and caring responsibilities. Ask which local supports are available; resources can be added to your record when confirmed.

Your records and additional help

Keep essential instructions available on paper.

The passport and wallet card

The three-page companion gives the essentials. Use the passport for the agreed plan, visits, test results, symptoms and questions. The wallet QR opens this exact treatment’s further information; it contains no personal clinical record.

Your urgent contact plan

Have the treating service enter and verify daytime, out-of-hours and backup numbers before relying on this pack. Keep the written contact and temperature instructions where you and a support person can find them.

If you prefer paper

Use the full printable handbook in its chapter order and write questions in the passport. You do not need to scan the QR or follow an internet link to use the essential treatment and safety information.

Questions worth asking

How many weekly carboplatin doses are planned?

This source plans 7 weekly AUC 1.5 doses during radiotherapy, with each dose subject to safety review. Radiation has its own number of sessions and appointment calendar.

What does AUC mean?

It is a target drug exposure used by the clinical team to calculate your carboplatin dose from kidney function. It is not a fixed milligram dose and is not a calculation you need to perform.

What if radiation is cancelled or stopped?

Contact both teams before the next carboplatin dose. This regimen is only given with radiation, and carboplatin must be discontinued if the radiation course is cut short. The teams confirm the revised plan.

Do I keep medicines from my previous chemotherapy?

Only if your team includes them in the new prescription. Induction steroids, growth factors, antibiotics and cisplatin fluids do not automatically carry forward.

When will my mouth and swallowing recover?

Recovery varies and effects can continue after radiation finishes. Keep pain, dental, nutrition and swallowing support and report worsening symptoms; the last session does not mean support should stop.

> **Try explaining the plan back:** Can I explain the planned weekly carboplatin count and the separate radiation calendar? Do I know whom to call if a radiation appointment changes before chemotherapy? Do I know when my blood tests are due and which symptoms need urgent contact? Can I explain my mouth, nutrition, swallowing and home medicine plans?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Seven weekly carboplatin AUC 1.5 visits after induction

Active medicines: Carboplatin

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	PRN Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Carboplatin AUC 1.5 intravenous (IV) every 7 days for 7 cycles, only while radiotherapy is being delivered. Discontinue carboplatin if radiotherapy is truncated.

- The treatment paragraph describes a start 3–8 weeks (Days 22–56) after the START of cycle 3 of induction; the indication paragraph says 3–8 weeks after COMPLETION of induction. The multidisciplinary team (MDT) must resolve the reference point and set the actual radiotherapy and chemotherapy dates. Confirm the induction route and adequate recovery.
- NCCP gives no routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or additional supportive-care prescription for this weekly phase. Do not carry over induction G-CSF, docetaxel premedication or cisplatin hydration automatically.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth, saliva and teeth

Head and neck radiotherapy

Soreness (mucositis), dry mouth and taste change may affect intake. Dental risk may persist; keep the oral-care and dental plan.

2 Throat, voice and swallowing

Head and neck radiotherapy

Pain or difficulty swallowing can build up and may last. Report choking, food sticking or inability to drink; ask for swallowing support.

3 Skin in the radiation area

Radiotherapy

Redness, soreness or skin breakdown may occur in the treated area. Use the radiation team's skin-care plan and report worsening pain.

4 Blood and infection

Carboplatin

Low white cells can cause serious infection; low platelets can cause bleeding; anaemia can cause fatigue. Fever, sudden illness or unusual bleeding needs urgent contact.

5 Stomach and intake

Carboplatin + radiation effects

Nausea, vomiting and poor intake can cause dehydration. Take prescribed anti-sickness medicines and call if you cannot drink.

6 Kidneys and fluid balance

Carboplatin

Kidney function determines the dose. Report reduced urine, vomiting or poor intake; do not calculate or change the dose yourself.

7 Hearing and ears

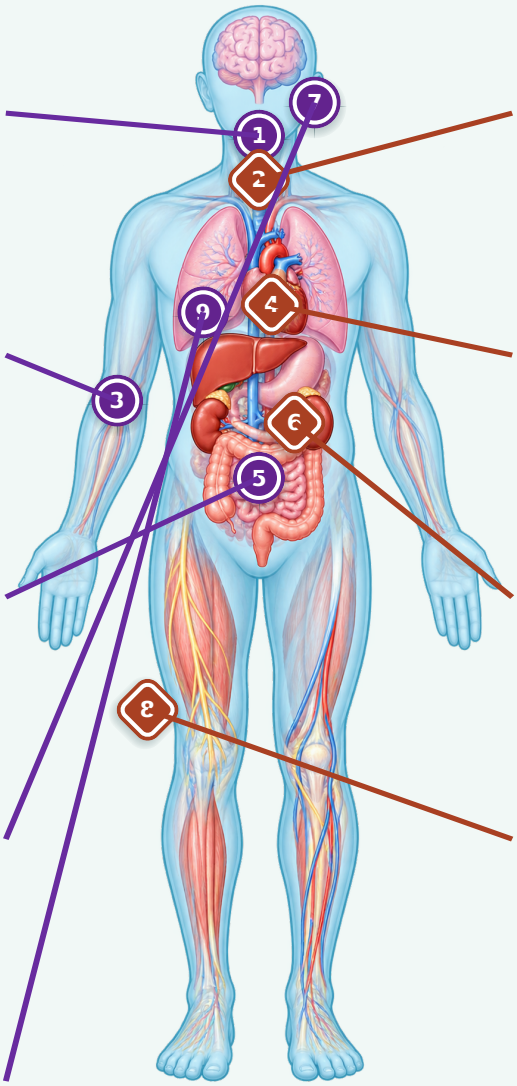
Platinum chemotherapy

Hearing change or ringing may be new or persist from earlier treatment. Report it before the next dose.

8 Hands, feet and nerves

Platinum chemotherapy

Numbness or tingling (peripheral neuropathy) may affect walking or fine tasks. Report changes and any earlier persistent symptoms.



9 Whole-body infusion reaction

Carboplatin

Rash, flushing, chest tightness, breathing difficulty or faintness may signal an allergic reaction. Tell infusion staff immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, unusual bruising or bleeding, persistent vomiting, severe mouth or throat pain, inability to drink or much less urine. Report new hearing changes, numbness, worsening swallowing, choking or skin breakdown promptly. During an infusion, tell staff immediately about rash, flushing, chest tightness, breathing difficulty or faintness. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112 or 999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00332 v4 · pNCCP 00332: treatment/supportive-care sections; NCIS v8 pp4–5**
   https://healthservice.hse.ie/documents/6959/332_V4_Carboplatin_AUC1.5.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP 00332 with current NCIS and local supportive-care policy**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf